

PROVISIONAL SUMMARY RECORD OF THE THIRD MEETING

**Palais des Nations, Geneva
Tuesday, 19 May 2015, scheduled at 14:30**

Chairman: Mr E. JARAMILLO NAVARRETE (Mexico)

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COMMITTEE A
THIRD MEETING

Tuesday, 19 May 2015, at 14:40

Chairman: Mr E. JARAMILLO NAVARRETE (Mexico)

COMMUNICABLE DISEASES: Item 16 of the Agenda (continued)

2014 Ebola virus disease outbreak and follow-up to the Special Session of the Executive Board on Ebola: Item 16.1 of the Agenda (Documents A68/24, A68/25, A68/26, A68/27, A68/51 Rev.1 and A68/56)

Introducing the item, the CHAIRMAN drew attention to the draft decision contained in document A68/51 Rev.1, which read as follows:

Further to the submission of the reports on 2014 Ebola virus disease outbreak and follow-up to the Special Session of the Executive Board on Ebola (documents A68/24, A68/25, A68/26 and A68/27), the Health Assembly is invited to consider the following draft decision.

The Sixty-eighth World Health Assembly, having recalled the resolution adopted by the Executive Board in its Special Session of 25 January 2015,¹

Interim assessment

- Welcomed the preliminary report of the Ebola Interim Assessment Panel appearing in document A68/25;
- Thanked the Ebola Interim Assessment Panel for its work to date;
- Requested the Ebola Interim Assessment Panel to continue its work, and to issue a final report to be made available to the Director-General not later than 31 July 2015.

International Health Regulations (2005)

- Requested the Director-General to establish a Review Committee under the International Health Regulations (2005) to examine the role of the International Health Regulations (2005) in the Ebola outbreak and response, with the following objectives:
 - to assess the effectiveness of the International Health Regulations (2005) with regard to the prevention of, preparedness for and response to the Ebola outbreak, with a particular focus on notification and related incentives, temporary recommendations, additional measures, declaration of a Public Health Emergency of International Concern and national core capacities;
 - to assess the status of implementation of recommendations from the previous Review Committee in 2011² and related impact on the current Ebola outbreak;

¹ Resolution EBSS3.R1.

² See document A64/10.

- to recommend steps to improve the functioning, transparency, effectiveness and efficiency of the International Health Regulations (2005) and to strengthen preparedness and response for future emergencies with health consequences, with proposed timelines for any such steps.
- Requested the Director-General to convene the International Health Regulations (2005) Review Committee in July 2015, and to report on its progress to the Sixty-ninth World Health Assembly in May 2016;
- Agreed to support west and central African States and other at-risk States to achieve full implementation International Health Regulations (2005) by June 2019;
- Endorsed the proposal of the Ebola Interim Assessment Panel that peer review or other equivalent external validation be part of any assessment of the status of national core capacities under the International Health Regulations (2005).

Global health emergency workforce

- Welcomed and concurred with the Director-General's conceptual plan for a Global Health Emergency Workforce to respond to acute or protracted risks and emergencies with health consequences;
- Requested the Director-General to report on progress on the establishment, coordination and management of the Global Health Emergency Workforce to the Executive Board at its 138th session in January 2016.

Contingency fund

- **[Welcomed the parameters described in document A68/26, which include the guiding principles that must govern the fund, such as: size, scope, sustainability, operations, sources of financing and accountability mechanisms;] (Panama)**
- Decided to create a **[specific,] (Finland) [revolving] (Egypt) contingency fund [merging the existing two WHO funds¹,] (Finland)** with a target capitalization of US\$ 100 million **[[in fully flexible funds] / [, fully funded by flexible voluntary contributions] (Spain, Portugal, Italy), [so as to provide enhanced response to all possible concurrent emergencies] (Egypt) to reliably[, transparently and in an accountable manner, including financial reporting, and applying the principles and practices of neutrality, humanity, impartiality and independence of good humanitarian donorship in its use,] (Finland) finance and rapidly scale up WHO's initial response[, including through other partners, (UK) to [[emergencies with health consequences] / [an escalating health emergency, using the objective criteria set out in the ERF]] (UK) for a period of [up to] three [to six] months [from the start of an emergency of grade 2² or higher and to address, if and when necessary, factors that could prevent the escalation of a given emergency or risk] (UK);**
- Decided that the Contingency Fund would be under the authority of the Director-General **[, or his or her delegate] (UK)**, with disbursement at his or her discretion;
- **[Requested the Director-General to establish a mechanism for monitoring and evaluation of the operations of the contingency fund.] (Panama)**
- Thanked Member States for contributions already committed to the Contingency Fund;

¹ [WHO's Rapid Response Account and WHO-Nuclear Threat Initiative Emergency Outbreak Response Fund.] (Finland)

² [As per the criteria articulated in WHO's Emergency Response Framework.]

- Requested the Director-General to approach donors to encourage contribution to the Contingency Fund, including through the next round of the Financing Dialogue;
- Requested the Director-General to report on the performance of the Contingency Fund, including amount raised and spent, and for what purpose, to the Sixty-ninth World Health Assembly in May 2016, through the Executive Board at its 138th session in January 2016.
- **[Requested the Director-General, in consultation with the Executive Board, to establish procedures for transparency and submission of accounts.] (Panama, Argentina, Mexico)**
- **Requested the Director-General to prioritize in-field operations in affected countries when using contingency fund. (Panama, Mexico)**

Research and development

- Appreciated the key coordination role played by WHO for ongoing work in development of vaccines, diagnostics and medicines for the Ebola virus disease;
- Endorsed the development of a framework for advancing research and development of medical products for other infectious diseases of epidemic potential, taking into account other relevant work streams within WHO.

Health systems strengthening

- Welcomed the development of the robust, costed national health system recovery plans for Guinea, Liberia and Sierra Leone, which were presented at the World Bank Spring Meetings on 17 April 2015, as the basis for donor coordination and strategic investments;
- Requested WHO to continue its coordination role in support of national administrations as they prepare for the United Nations Secretary General's high-level pledging conference on Ebola, to be held on 10 July 2015;
- Acknowledged the leadership shown by the Ministries of Health of the three countries in focusing, with support of WHO country offices, on early recovery through emphases on infection prevention and control, reactivation of essential services, immediate health workforce priorities and integrated disease surveillance;
- Requested the Director-General to continue and enhance the work of the Organization in supporting Member States to be better prepared to respond to emergencies with health consequences by strengthening national health systems.

Way forward

- Welcomed the Director-General's commitment to reform the work of WHO in emergencies with health consequences;
- Requested the Director-General to report on progress on these reforms, and on the other decisions taken herein, to the Executive Board at its 138th session in January 2016.

Ebola Interim Assessment Panel (Document A68/25)

Dame Barbara STOCKING, Chairman of the Ebola Interim Assessment Panel, said that the Panel's report was intended to highlight, as a matter of urgency, all the lessons learned or to be learned from the Ebola crisis. It would be followed by a final report in July 2015. The Panel had focused on three main areas in its work, namely the International Health Regulations (2005); the development of WHO, in the sense primarily of the Secretariat, as an emergency agency and the links between the health emergency system and the wider humanitarian system. Firstly, the Panel was convinced that if the recommendations contained in the Report of the Review Committee on the Functioning of the International Health Regulations (2005) in relation to Pandemic (H1N1) 2009 had been implemented, there would have been a far better basis for management of the outbreak of Ebola virus disease. Urgent action was needed to enhance Member State preparedness. That would require funding, and WHO should work with the World Bank in that respect. Action was needed in many areas, including

the implementation of outside assessment of countries' surveillance systems in order to increase trust in them; strengthening of capacity for community engagement; data aggregation across agencies, using common definitions and standards; and a more progressive classification of public health emergencies. There was a need for incentives, such as the reinsurance mechanisms proposed by the World Bank, to encourage countries to declare outbreaks as soon as possible; at present major disincentives existed, such as the threats to the economy, transport and trade which would arise from declaration of an outbreak. In response to the Ebola virus disease outbreak, over 40 countries had introduced more restrictive protective measures than WHO had suggested, including trade and transport barriers. WHO should have more powers in that regard, but additional mechanisms should be investigated, such as sanctions through the World Trade Organization.

Welcoming the commitment of the Secretariat, as expressed by the Director-General, to position WHO as an emergency agency for all aspects of health crises, the Panel endorsed her concept of a single entity within WHO to bring together the International Health Regulations (2005) issues and the emergency response issues, but considered that creating a separate agency was not a sensible approach. Setting it up would be time-consuming and costly and emergency response was a logical component of WHO's core mandate of managing global public health. Nevertheless, it was important to be aware of the lack of public trust in WHO resulting from the Ebola virus disease crisis; a significant cultural change within the Organization to do with handling emergencies would be needed. Accountability mechanisms, to monitor the change efforts of the Secretariat, would also be a vital component, as would the establishment of deeper partnerships with other agencies. In terms of links between the public health system and the United Nations emergency response architecture, although WHO was the lead agency in the health cluster, the two sides had only limited understanding of the other's roles and activities. The Panel intended to highlight the issue of greater mutual comprehension in discussions with the United Nations Secretary-General's High-Level Panel on the Global Response to Health Crises.

Dr JUAN LOPEZ (Mexico) welcomed the work and recommendations of the Ebola Interim Assessment Panel, particularly in terms of identifying areas for improvement both at the country level and within the Secretariat. Mexico had developed an action plan to tackle such outbreaks, as had many other countries, and the results of the independent assessment would be an extremely useful contribution.

Ms RABOVICA (Latvia), speaking on behalf of the European Union and its Member States, said that they remained fully committed to reaching zero cases, helping the affected countries recover and strengthening the preparedness of those and other countries. The Panel's first report provided very pertinent recommendations for improving WHO's emergency response, as well as accurately describing the gaps and delays in WHO's response in the early stages of the crisis, and she looked forward to further analysis in the Panel's final report. She would hope for more focus on the relationship between WHO and other partners in the field, together with further analysis of WHO's management of the foreign medical teams. She welcomed the emphasis given by the Panel to improving implementation of the International Health Regulations (2005). That was of particular importance for outbreaks in vulnerable countries, which had limited capacity to respond effectively without additional support. However, it was important that proposals did not go beyond the remit of the Regulations. Noting the report's recommendation concerning a cultural and organizational shift in emergency response within the Organization, she said that the ongoing reform of WHO should help to close the gaps in leadership and crisis coordination. The European Union and its Member States were undertaking a review of lessons learnt from the Ebola virus disease outbreak, examining their own preparedness and response. Finally, the conclusions of the Panel should feed into the deliberations of the United Nations Secretary-General's High-Level Panel on the Global Response to Health Crises.

Dr AMMAR (Lebanon) said that WHO should be better prepared and equipped for public health events of international concern; moreover, many Member States required the urgent support of the Organization to accelerate their preparedness and to build the core capacities stipulated by the International Health Regulations (2005). Independent assessment of capacities in that regard, such as that conducted by the Regional Office for the Eastern Mediterranean in some countries in the Region, would enhance Member States' accountability at both the country and global levels. In emergency situations, exceptional measures should be permitted in order to overcome WHO bureaucratic bottlenecks and ensure a rapid and appropriate response. As noted by the Panel, WHO suffered from a lack of political and financial commitment from its Member States, despite global health risks. The continuing policy of zero nominal growth was a key example: it was time to revisit that policy and to create a core fund for emergency response. The countries of the African and the Eastern Mediterranean Regions had already indicated their readiness to accept an increase in assessed contributions to that end.

Dr HINOSHITA (Japan) observed that it was essential to have in place and to have rehearsed an emergency response plan; capacity-building for health care workers, particularly at the country level, was indispensable in that regard. The proposed contingency fund should be efficient and take other relevant funding mechanisms into account. Also, it would be important to clarify the difference between the activities supported by the contingency fund and other existing and future programmes. Lastly, he acknowledged the importance of strengthening core capacities as stipulated by the International Health Regulations (2005), in order to build resilient health systems.

Ms LANTERI (Monaco), stressing the importance of the Panel's report in identifying and rectifying the problems and failures that had arisen during the Ebola virus disease outbreak and that could recur in future, said that it was essential to work together and undertake measures to enable Member States to react to situations as effectively as possible and to empower WHO to fulfil its mandate. She requested further information on the interactions between the Ebola Interim Assessment Panel and the United Nations Secretary-General's High-Level Panel on the Global Response to Health Crises.

Mr STAUR (Denmark) said that his delegation fully supported the Director-General's endeavours to ensure that WHO played its rightful role in disease outbreaks, humanitarian emergencies and global health security issues. WHO must ensure that it had robust emergency operations in place and was fit for purpose and accountable. He called on the Director-General to lead the substantial changes across all levels of the Organization that would be required. He looked forward to the Interim Assessment Panel's final report, including a clear indication of what had and had not worked at each level of the Organization. Meanwhile, he supported the plan for a global health emergency workforce and the proposed framework for a contingency fund.

Dr BOKENGE BOSUA (Democratic Republic of the Congo) recalled that the Democratic Republic of the Congo had dealt with six Ebola virus disease outbreaks since 1976. The latest had been in August 2014 and had been brought fully under control in 45 days by means of an experience-based national strategy in line with the International Health Regulations (2005), relying on community vigilance, multisectoral coordination, strengthening of diagnostic capacities and the deployment of multidisciplinary teams.

Dr ENNIS (Jamaica) said that WHO should as a priority provide increased support to countries to assist them in achieving the International Health Regulations (2005) core surveillance and response capacity requirements. At present, there was a lack of confidence in the system, as the tools to respond were non-existent. It was essential that the Region have a stockpile of personal protective equipment, to which end WHO must establish a regional hub. Jamaica and other countries did not have the human

and financial capacities to adequately deal with an outbreak of the magnitude of the recent Ebola virus disease. Therefore, the plan to expand the global health emergency workforce and establish a contingency fund should be swiftly implemented. WHO must rebuild the trust of its Member States, which could be done through inaugurating a more organized humanitarian and emergency response, with timely and clear decision-making. Jamaica supported the actions recommended by the Panel and looked forward to the final report.

Dr NOHA SALAH (Egypt) said that since the beginning of the Ebola virus disease outbreak, Egypt had had measures in place to deal with it and had provided help to some African countries. The comments made in an assessment by WHO showed no appreciation for the steps taken by the Egyptian Ministry of Health and other authorities, and Egypt understood that the same was happening in several other countries. WHO was transferring the responsibility for preparedness and response to its Member States alone, whereas it should be a joint effort of the Secretariat and the Member States together. She asked whether there would be repercussions for Member States that did not implement the International Health Regulations (2005).

Mr AL SHARQAWI (Bahrain), speaking on behalf of the Eastern Mediterranean Region, said that it was in the best interests of the global community to have a strong and responsive WHO, with the right resources and means to address international health threats adequately and effectively. WHO must reform its existing emergency response structure and address its organizational weaknesses. His delegation endorsed the conclusion of the IHR Review Committee's assessment of the Ebola crisis with regard to weaknesses and gaps in the notification and information-sharing process.

Dr ASSIRI (Saudi Arabia) said that as WHO's capacity to respond effectively to multiple public health emergencies remained constrained, the recommendations of the Ebola Interim Assessment Panel needed to be considered seriously. The required core capacities under the International Health Regulations (2005) must be achieved and sustained. The proposed IHR Review Committee assessment of the Ebola crisis should examine the weaknesses and gaps and make recommendations for improvements.

Mr GULDVÅG (Norway) said that the report of the Ebola Interim Assessment Panel provided a convincing analysis of structural shortcomings in need of urgent correction. It was the responsibility both of the Member States and of the Secretariat to fulfil the provisions of resolution EBSS3.R1. Member States needed to provide the resources and the political backing to enable the Director-General to make the necessary changes, to continue to develop her concepts for reform and to implement them throughout the Organization. With the outbreak having demonstrated the Organization's lack of capacity to handle complex emergencies, he welcomed the commitment to address such shortcomings, emphasizing that structural changes must accompany the reform elements decided at the Special Session of the Executive Board on Ebola. Effective crisis management required information-sharing systems across the Organization as well as mechanisms for quick decision-making and unified lines of command. WHO should also cultivate partnerships with other United Nations system entities, non-State actors and the private sector.

Dr AARABI (Islamic Republic of Iran) emphasized that in order to address future epidemics of international concern, a strong global health system was needed with the capacity to detect outbreaks early. Rapid response teams at global, regional and national levels must be created. Military forces could assist with logistics and transportation, as well as securing the area. The Secretariat and the Member States should organize simulations and drills to help identify performance gaps. They also needed to foster advanced research into and development of vaccines, medicines and diagnostics, which would not only improve global preparedness against public health emergencies of international concern but would also strengthen primary health care systems and reduce the inequities in health

services. Member States should prioritize surveillance, preparedness and response capacities. Additionally, the more advanced Member States should share their experience and capacities with less developed countries, as Iran was prepared to do.

Dr JABER HUWAIL (Iraq) said that the Ebola virus disease outbreak had tested the robustness of the International Health Regulations (2005), the health system and partnership development. Emphasis should be placed on the effectiveness of the Regulations; the role of WHO in coordinating activities related to the Regulations; and the roles of other, non-health, sectors. The approach to funding should include strengthening budget transfers within regular programmes for emergency preparedness and response and following a carefully prepared contingency work plan for investing the resources of WHO and other international organizations. More generally, the focus should be on strengthening health system building blocks and the relation of funding with universal health coverage, including emergency preparedness and response.

Dr FERNANDEZ MUÑOZ PORRAS (Chile) welcomed the initiatives aimed at improving countries' capacities in order to avoid or diminish the effects of health crises such as the Ebola disease virus outbreak. The International Health Regulations (2005) helped align countries to create a safer world, but their effectiveness was proportionate to Member State compliance. In view of the difficulties expressed by many countries in implementing the capacities within the established time frame and given the practical and logistical problems which increased the risk of Ebola disease virus importation, a global early detection and prevention effort was needed, based on collaborative support between Member States and with international organizations. He supported the creation of a contingency fund for emergencies. His country's recent emergencies and the threat of the Ebola virus disease had demonstrated how important it was to have a functioning system for assessing detection, analysis, notification and response capacities. Urging that current efforts be strengthened in order to share the lessons learned from the crisis among Member States, he supported the conclusions of the report and the implementation of the recommendations.

Mr CARIKCI (Turkey) said that Member States could strengthen WHO capacities during emergencies and outbreaks by providing human resources, financing, partnerships and communication capabilities. While much had been achieved in dealing with the Ebola virus disease outbreak, it was important to stay vigilant in order to reach the target of zero cases. Preparedness and resilience were key elements of response to emergencies. A fully-fledged resource mobilization effort was needed in order to achieve those two elements, with a focus on building resilient health systems and strengthening primary health care and universal health coverage. The capacity-building efforts in fragile countries should be supported through strong partnerships. Urging that the affected countries not be economically or socially isolated, he described the assistance that Turkey had provided and would continue to provide to them.

Mr KONYNDYK (United States of America) said that his country greatly appreciated the report of the Ebola Interim Assessment Panel, which provided a compelling analysis of the underlying causes for the weakness of WHO's initial response to the Ebola virus disease outbreak. He noted that while the United Nations Mission for Ebola Emergency Response had attempted to identify appropriate linkages and burden-sharing between the global health and humanitarian response communities, that might not be the best model for organizing future responses to global health crises. To address those shortcomings, a strong and empowered emergency response section within WHO should be developed. A clear and unified command and control structure was central to effective emergency management. He endorsed the report's observation that self-reporting on implementation of the International Health Regulations (2005) was not adequate and the scope of the Regulations was not large enough to protect against inappropriate international travel and trade restrictions. He strongly

supported an external validation framework to help accelerate complete compliance with the Regulations.

Dr MMBANDO (United Republic of Tanzania) said that the Ebola outbreak had tested the health systems of developing and developed countries alike, with panic being expressed by all nations. He called on WHO to support countries in building a robust surveillance and response system as part of resilient health systems, address Ebola and other neglected diseases in a holistic manner and assist countries that were building laboratory capacity for emerging and re-emerging diseases. He welcomed WHO's proposals to revisit the global health infrastructure, at all levels, examining coordination and finances, and noted the importance of building countries' core capacities to ensure implementation of the International Health Regulations (2005).

Dr E. M. NDIAYE (Senegal) welcomed the report.

Dr GONZÁLEZ FERNÁNDEZ (Cuba) asked when the final report of the Ebola Interim Assessment Panel would be made available to Member States. The International Health Regulations (2005) were a useful tool and more emphasis should be placed on developing core capacities in all Member States. Strong emphasis should be placed on epidemiological surveillance, community participation and the involvement of all sectors, contributing to the ability of each country to detect a public health emergency of international concern. He described the steps taken in Cuba and other countries of the Region to tackle Ebola or similar outbreaks. The capacity of WHO to confront emergency situations and draw lessons from them should be increased. He agreed that greater synergy was needed between the broader United Nations system and WHO, with the latter continuing to take the lead in health-related activities.

Professor DELFRAISSY (France) said that lessons must be learned, and conclusions drawn as to the work of WHO, from the crisis. The gains made were due to having a robust strategy and appropriate human and financial resources; cooperation among all actors, in particular the local communities affected by the epidemic, was crucial to maintaining those gains. Member States had learned that they needed to be better prepared for major health emergencies, which had an impact on many areas in addition to health. The Assembly should work to introduce a contingency fund and financially sustainable and robust health reserves for rapid mobilization. His country would participate actively in those discussions. He described the assistance being provided by France to Guinea, one of the affected countries. Research into tropical and emerging diseases was crucial, and epidemiological surveillance should be coordinated at regional level, since disease did not recognize borders. The effective implementation of the International Health Regulations (2005) was a fundamental step in creating resilient health systems as part of a broader sustainable development goal. His country would continue to support WHO in responding to health crises as the lead of the United Nations health cluster, and to support reform that would increase effectiveness.

Dr MELNIKOVA (Russian Federation) supported most of the recommendations made by the Panel. In order to strengthen the emergency response to crises like Ebola, react adequately and effectively to emergency situations, response management crises should be centralized under the auspices of WHO and the use of Member States' resources should be optimized. WHO must also work more effectively on coordinating those resources. Member States' resources, including staff, laboratories, transport and medical supplies should form the basis of a global reserve for responding to public health emergencies. There should be a clear unified method for mobilizing Member States' resources in crisis situations, and her country stood ready to contribute specialist staff and resources to an emergency response led by WHO. She welcomed the Panel's assertion that the main work, such as strengthening countries' ability to respond, should be performed between crises, not during them. She supported the establishment of a contingency fund based on voluntary additional donations from

Member States and operating efficiently and transparently. The Russian Federation would continue its active international participation in efforts to tackle Ebola, to which it had already contributed more than 60 million US dollars, and would maintain support for affected countries in Africa once the outbreak had come to an end.

Ms DUSSEY-CAVASSINI (Switzerland) welcomed the report of the Ebola Interim Assessment Panel, as an analysis of the response to Ebola was important, both to WHO itself and to the United Nations system. She encouraged coordination between the Ebola Interim Assessment Panel and the High-Level Panel on Global Response to Health Crises. Efforts to strengthen health systems and the implementation of the International Health Regulations (2005) should be intensified and she welcomed the establishment of the IHR Review Committee. WHO should take its due role of leadership of the health cluster in humanitarian response, assigned to it more than a decade previously. A change in culture would be needed within the WHO unit responsible for managing emergency response, since crisis management was far different from development of standards or directives. WHO must develop adequate professional competencies to provide a flexible response to health emergencies. When responding to emergencies there was no place for competition within the United Nations system and, while the capacity of WHO should be strengthened, there should be no duplication of work performed elsewhere in the system. She welcomed the planned conclusion of agreements between WHO and other agencies, both within and outside the international system, preferably before any crisis occurred.

Mr KUEMMEL (Germany) remarked on the importance of learning from the mistakes of all, including WHO, in order to strengthen the Organization's capacity and allow it to regain its credibility and role in global health. The Panel should address the importance of better understanding the role of WHO country offices in the event of disease outbreaks. Member States bore much of the responsibility for WHO's emergency response capacities, since the resources underpinning the Secretariat's capacity to monitor International Health Regulations (2005) implementation and provide related technical support had been reduced to a level that was now inadequate. The policy of zero nominal growth in assessed contributions had clearly eroded the work of the Secretariat, and Member States' financing of the Organization had contributed significantly to its failure to meet expectations. Making WHO fit for purpose in the area of emergency response would require major investments and firm political will on the part of all Member States. He strongly supported the Panel's key conclusion that the present was the moment for empowering WHO to lead in global public health.

Mr ZHANG Guoxin (China) supported the establishment of a contingency fund and a global health emergency workforce and his country stood ready to participate in those initiatives. He suggested that, instead of making the effective implementation of the International Health Regulations (2005) a priority, Member States should instead be helped to improve their capacity to cope with public health emergencies, including by establishing and improving robust public health systems in Africa.

Dr AHMED SIDAHMED MOHAMMED (Sudan) noted that her country's previous experience of Ebola had encouraged it to develop a preparedness plan when the Ebola outbreak had been announced, largely based on International Health Regulations (2005) core capacities. The fight against epidemics should start with increased financial and technical support for countries like hers that had asked for a further extension of the implementation deadline for the Regulations. Since rapid, decisive action was required from WHO in emergency situations she welcomed the establishment of a specific programme to deal with health emergencies within WHO, and the development of a contingency fund. While a global health emergency workforce was a key issue, since most countries affected by health emergencies had a severe shortage of human resources for health, she stressed the parallel importance of developing the capacities of national health personnel.

Dr PHAAHLA (South Africa), speaking on behalf of the African Region, said that Ebola presented one of the biggest public health challenges experienced in Africa and around the world and that the high rates of infection among health workers and the consequent impact on health systems were a particular cause of concern. The situation had nonetheless improved, due in large part to the swift response and bold leadership of some countries of the African continent. He expressed the hope that Member States appreciated the impact of the Ebola outbreak on health systems in Africa and would offer assistance to strengthen those health systems.

Mr DE ANDRADE FILHO (Brazil) said that, although health was the responsibility of States, international collective action was needed to respond to health threats at global level. Global preparedness should be ensured by implementing the International Health Regulations (2005). He supported a central role for WHO in the timely response to global public health emergencies and noted that the Organization would be as effective as the synergies and coordination it established with other partners, particularly other United Nations agencies. That work should be undertaken while strengthening the leadership of WHO as an independent source of technical and normative health standards and recommendations. The introduction, during the Ebola outbreak, of measures that interfered with travel and trade and that were contrary to the recommendations set forth by the Director-General on the advice of the IHR Emergency Committee was a cause for concern that should be addressed in discussions on future disease outbreak responses. Existing incentives for research and development had not met the need for research and development of diagnostics, vaccines and treatments for Ebola. An expanded and trained health emergency workforce should be reinforced by partnerships with different actors and be guided by the interests of public health.

Ms OUTHWAITE (Canada) supported the leadership role of WHO in managing global public health events. The present was a defining moment for WHO, entailing a collective responsibility to address weaknesses in health crisis prevention and response. While it was important to establish stronger reporting incentives for the International Health Regulations (2005), a clear and robust policy basis was also needed to address the question of border measures raised by the Panel. Steps should be taken to ensure that WHO was strengthened and appropriately structured to lead the international response to emergencies with public health consequences.

Dr GOMEZ (Bahamas), speaking on behalf of the Caribbean Community, said that maritime trade and porous borders contributed to the vulnerabilities of some of the countries of the Region of the Americas in the event of global public health emergencies. As of 2014, no country in the Region had met the core requirements of the International Health Regulations (2005). He therefore welcomed the announcement that those regulations would be re-examined, which would provide an opportunity to address the core capacity requirements of small island developing States. He requested that the Director-General should pay special attention to strengthening the health systems and core capacities of the countries of the Caribbean Community, in order to prepare for public health emergencies of international concern.

Professor NASIDI (Nigeria), recalling that his country had been one of the first to be affected by Ebola, described the steps taken to tackle the disease in Nigeria and then, after being declared Ebola-free, to move on to assisting the other affected countries. He welcomed the findings of the Ebola Interim Assessment Panel's report and called on all nations to support efforts in Africa to establish an African Centre for Disease Control (CDC). The Centre would not replace WHO but would apply the partnership model established between PAHO and the CDC in the United States.

Dame Sally DAVIES (United Kingdom of Great Britain and Northern Ireland) welcomed the insightful assessment and recommendations made by the Ebola Interim Assessment Panel, in particular those highlighting significant gaps in WHO's response and recognizing that the public

health emergency of international concern should have been declared earlier. The United Kingdom agreed with the report that greater consideration should be given to different levels of alert and that WHO outbreak's and humanitarian emergency response activities should be merged. She welcomed the recommendation to boost WHO response capacity, including by establishing a global health emergency workforce, but stressed that such measures should be aligned with existing cluster arrangements within the Organization's humanitarian mandate.

Dr TILLUCKDHARRY (Trinidad and Tobago) said that the threat of the Ebola virus disease had provided an opportunity to operationalize the International Health Regulations (2005) and assess their effectiveness. He therefore supported the establishment of a review committee as a step towards building trust and commitment. He also supported the extension, without stigma, of the target date for implementation of the Regulations, which should apply to any Member State still facing implementation difficulties. His country was committed to engaging in humanitarian action and supported proactive resource mobilization which was not necessarily Ebola-specific, such as the proposed contingency fund. He strongly advocated strengthening national, regional and global health security infrastructures.

Dr AHMED (Bangladesh) said that the crisis had raised awareness regarding the lack of resilience in health systems in certain countries. He outlined the response actions taken in Bangladesh following the declaration of the emergency and called on WHO to mobilize more resources to prepare States for such emergencies.

Dr ROA (Panama) agreed with other speakers that there was a need for adequate funding to enable WHO to manage health emergencies rapidly and effectively. A reserve budget should be established to provide immediate operational support in countries where need was increasing owing to a health emergency. The contingency fund should be financed by voluntary contributions and abide by the principles established under document A68/26, including measures to ensure transparency and accountability, and should be reviewed after two years.

Dr DANG QUANG TAN (Viet Nam) said that the review of the response to the Ebola virus disease outbreak would provide valuable lessons for future actions. He suggested that WHO should develop a comprehensive plan and should undertake the necessary reforms to prepare for future emergencies. Further, the Organization should maintain and enhance its partnership capacities with United Nations agencies and other organizations in preparedness planning and response. Member States should strengthen their health systems and core capacities as required by the International Health Regulations (2005) and he called on WHO to continue providing support to that end. In view of the importance of regional and global collaboration in emergency responses, he supported WHO's initiative to establish a joint team to support affected countries in Africa.

Dr AZZOUZI SIDI (Morocco) welcomed the report and the recommendations of the Ebola Interim Assessment Panel. He was convinced of the need for a strong WHO, in terms both of the qualifications of its staff and its resources. There was a clear need for political commitment on the part of all its Member States, as well as for reinforced capacities to implement the International Health Regulations (2005). In health crises, humanitarian considerations were paramount. Urging that the problem of cross-border transmission be addressed in that light, with the aim of isolating diseases rather than countries, he described how Morocco had sought to implement that principle.

Dr GUERRA (Italy) said that a skilled and competent WHO was crucial to the achievement of a truly global response to health emergencies. The Organization could not be left to work in isolation but was called on to complement national capacities. Responses should go beyond simply increasing financial resources, and involved deciding how resources would be used, developing partnerships with

other stakeholders, and ensuring a competent workforce was available when needed. The role of WHO was to promote research, develop standards, and provide authoritative and timely coordination. He highlighted the importance of a regional response to global health crises, and the need to strengthen WHO's organizational levels accordingly. The Organization's credibility needed to be strengthened and, in that light, he welcomed the review exercise. The results should be used to establish a better and more transparent response system and a more resilient WHO.

Professor LEVENTHAL (Israel) said that the Ebola crisis shed light on contradictions in the expectations that the Member States had of WHO. If they wished it to remain small and efficient, they should act as its extension in the deployment of manpower, equipment and medicines. A chapter should be added to the International Health Regulations (2005) to give WHO the leadership and coordination role in any future emergency response, such that the Secretariat and the Member States would act as one organization. Expressing his support for the Director-General and her leadership of the Organization, he called for the same level of support from all Member States.

Mr BOWLES (Australia) said that the report of the Ebola Interim Assessment Panel should give the 138th session of the Executive Board a more complete picture of how WHO's reforms had to strengthen its emergency response structures. He agreed with an earlier speaker that border measures under the International Health Regulations (2005) needed to have a strong policy basis. Member States and WHO needed to learn from the Ebola virus disease outbreak and use it as an opportunity to create a stronger health security system. Establishing a contingency fund and a global health emergency workforce were important elements to ensure WHO's response capacity. He called for further discussion on the governance of a global health emergency workforce and encouraged Member States to provide flexible funding, the lack of which had been one of the main limitations in WHO's response to the outbreak. Prevention activities through the contingency fund should be focused on preventing grade two emergencies progressing to grade three emergencies. Normal prevention and preparedness activities should be funded through the programme budget. WHO's plans to work in partnership with other organizations highlighted the importance of finalizing the framework for engagement with non-State actors. He welcomed the Organization's commitment to strengthening medical staff evacuation processes.

Ms CABELLO SARUBBI (Paraguay) said that the declaration of the Ebola virus disease outbreak had prompted a review of her country's capacity to deal with health emergencies, which had highlighted limitations in timely investment of resources and equipment. She supported the establishment of a contingency fund. She requested WHO to develop a plan to monitor the International Health Regulations (2005) after 2016 to ensure the maintenance of previously achieved core capacities, and a flexible and adaptive response to emergencies.

Mr PUSP (India) expressed support for the Panel's recommendations. Consideration should be given to strengthening emergency preparedness by establishing three levels of alert at the national, regional and international levels, entailing clear responsibilities for Member States and the three levels of the Organization. He agreed with the Panel's conclusion that WHO needed to be strengthened both structurally and financially to improve its response capacities. His Government was committed to supporting the proposed contingency fund.

Dr YONGJHUA LAOSIRITAWORN (Thailand) suggested that WHO should continue supporting research on the Ebola virus disease, focusing on epidemiology, the natural course of the disease, the mode of transmission and the strategy and model of prevention and control, covering scientific and cultural aspects and taking into account the new tools for diagnosis, prevention and treatment. He expressed broad support for the contingency fund, noting that adequate financing was

essential in prevention and control activities. It was crucial to ensure good governance in managing such a fund, with regular reporting to Member States.

Ms ALARCÓN MAYORGA (Colombia) said that, considering that WHO already had a mandate to provide an operational response to public health emergencies, it would be much more efficient to integrate a full response capacity within its existing structure rather than creating a new agency. Upgrading WHO's response capacity in this way would require profound institutional changes, and measures to ensure transparency and accountability. The Ebola outbreak had generated enormous pressure on national and international response capacities, highlighting the need to enhance surveillance and community engagement, to improve coordination with the UN system and to take national complexities into account, particularly in fragile states. The International Health Regulations (2005) provided the main framework for strengthening world health security. However, the means of integrating health emergency measures in the humanitarian system needed to be addressed. She called on WHO to take measures to improve its ability to participate in partnerships (which must be established before, not during, the emergency), and on Member States to strengthen their crisis response systems.

Dr LOGAN (Liberia) expressed his Government's support for the concept of a contingency fund for immediately addressing health emergencies, particularly since Liberia had been the victim of a gravely delayed response to the Ebola virus disease outbreak, resulting in the loss of thousands of lives, including those of almost 200 health care workers. It was crucial to close the gap between alert and response as the success of each system depended on the strength of the other; in the case of Ebola, the alert had come months too late.

Dr AYLWARD (Assistant Director-General) acknowledged Member States' expressions of encouragement to the Secretariat to build the capacity and cultures necessary to deliver large-scale humanitarian response, including work on capacity-building and verification of core capacities. He said that the Secretariat had also noted the firm call for a stronger WHO to operate as the cluster lead in large-scale humanitarian emergencies. He had taken note of the support for the contingency fund and the global health emergency workforce and the emphasis on the need to strengthen partnerships both with civil society and with bodies in the United Nations system. Above all, the discussion had highlighted strong agreement with the direction given to the Secretariat by the Interim Assessment Panel as to needed reforms.

The DIRECTOR-GENERAL expressed appreciation for the work of the Interim Assessment Panel, which had achieved a great deal in a short period. It would receive continuing support from the Secretariat to complete its activities by July, and its recommendations would be taken very seriously. Indeed, change had already started, notably in the area which was within the Director-General's managerial responsibility. The Interim Assessment Panel had further work to do, and Member States would undoubtedly wish to comment on the special resolution arising out of the January session of the Executive Board. All such comments would be considered, and the process of reform would continue.

DAME Barbara STOCKING (Chairman of the Ebola Interim Assessment Panel), thanked Member States for their support for the Panel's work; the Panel had taken note of their comments and reservations or requests for further information or evidence. She had been heartened by the commitment shown to strengthening the International Health Regulations (2005), particularly in the area of public health surveillance in-country, as well as by Member States' support for a strong, unifying WHO as a global leader, fit for purpose. Reforms could be made quickly now. The Panel would welcome the incorporation of its conclusions into other reviews worldwide, notably the United Nations Secretary-General's High-Level Panel on the Global Response to Health Crises.

The CHAIRMAN proposed that the remainder of the discussion should cover any or all of the outstanding Ebola-related sub-items.

Mr KONYNDYK (United States of America) said that the plan on the establishment of a global health emergency workforce was one of the most important ways of making WHO more fit for purpose and improving collective health security. Rather than creating a new entity, the workforce should be formed by building on, strengthening and expanding existing platforms, such as the Global Outbreak Alert and Response Network and the Foreign Medical Teams system. The effort should also explore how better to engage the capacities of personnel from the global humanitarian system. The United States supported the plan, which responded to all of the elements requested by the Executive Board through its resolution EBSS3/R1, in particular the need for a strengthened and dedicated support structure at WHO headquarters. He was pleased to note that the plan provided for training and capacity-building, which over time would enlarge the workforce. Another important factor would be the trigger mechanisms, but he pointed out that a decision to deploy the workforce in emergencies with health consequences would not always rest solely under the Director-General's authority, for example in cases where the lead was taken by the United Nations cluster system. Early deployment in response to infectious disease outbreaks would improve surveillance and contribute to WHO risk assessments. While regional and country offices had a key role to play in WHO's emergency preparedness and response and must be strengthened accordingly, the effectiveness of a global health emergency workforce depended on the ability of WHO headquarters to mobilize it quickly and decisively, through a clear line of authority starting with the Director-General. Finally, while WHO had an important convening and coordination role to play in research and development, which required that it enhance its expertise in clinical trials and that it remain neutral, his Government was concerned that WHO had become a principal investigator in specific trials associated with the Ebola response.

Mr ROUSHDY (Egypt), speaking on behalf of the Member States of the Eastern Mediterranean Region, expressed support for the contingency fund, which would give WHO the necessary capacity to mount a stronger and more vigorous response, especially during the early stages of an emergency or crisis. Two issues should, however, be addressed: namely, how to make such a fund sustainable, and how to ensure that it would be replenished quickly as money was disbursed. Since WHO often faced multiple emergencies simultaneously, money must be distributed quickly to wherever it was needed. In order to ensure accountability and transparency in the use of funds, an operational mechanism should be developed through broad-based consultation with WHO regional offices.

Ms RABOVICA (Latvia), speaking on behalf of the European Union and its Member States, hailed the plan for a global health emergency workforce as an ambitious step towards addressing current shortcomings in the global health system. The European Union assumed that the costs incurred would be included in the programme budget 2016–2017 and that the contingency fund, once established, could also be drawn on in emergencies. A clear and effective trigger mechanism would need to be designed as part of the operationalization of the global health emergency workforce. The European Union supported the broad scope of emergency response proposed by the Secretariat. Improved leadership and coordination within WHO would be key to raising the all-hazards surge capacity for rapid deployment. In addition, the European Union supported separate administrative and technological systems for ensuring rapid deployment in emergencies. Preparation should be enhanced, for example through advance agreements with partners on the main roles of teams on the ground. Existing networks and partnerships such as the medical evacuation system (MEDEVAC), established during the Ebola outbreak by the European Union and other partners in collaboration with WHO, needed to be strengthened. Under the European Union Civil Protection Mechanism, the European Union was also developing a voluntary pool of medical experts and response capacities which could

be drawn on for fast emergency response. It welcomed the proposal for a steering committee to ensure coherence between networks and partners, and it was prepared to play a constructive role in that effort.

Mr LUTZOW STEINER (Mexico) stated that Mexico supported the creation of a global health emergency workforce and offered to share with WHO its experience and expertise. Mexico was also in favour of the creation of a WHO contingency fund for emergencies, which should be transparent and sustainable. The fund should draw on as many sources of voluntary financial support as possible and be deployed in conjunction with other regional funds.

Mr KUEMMEL (Germany), speaking on behalf of the European Union and its Member States, expressed support for the creation of the contingency fund financed by flexible voluntary contributions, which would complement other funds both inside and outside WHO. The fund should be merged with existing disused funds, as outlined in document A68/26. The contingency fund would serve as a first source of funds, filling the gap created before other resources became available, for example from appeals. A three-month period should provide enough time for appeal funding to be made available. Disbursement of the fund should be at the discretion of the Director-General, with minimal bureaucracy to ensure rapid deployment, including delivery through partners. The European Union agreed with the recommendation of the IHR Review Committee that the fund should contain US\$ 100 million. In order to ensure transparent and accountable use of the fund, regular financial reporting would be required in addition to robust accountability structures, including full access to the fund's financial, implementation and performance data. The European Union proposed that the present Health Assembly should establish the fund on a pilot basis, with a review after two years.

Ms MOE (Norway) acknowledged that although the primary responsibility for preventing and responding to health emergencies remained with governments, sometimes external assistance was necessary. Norway supported many of the proposals presented in the Director-General's report on the global health emergency workforce, including improving early identification by Member States, providing regular training and simulations, developing a single emergency training system for WHO-driven deployments, and deepening partnerships with United Nations agencies and similar bodies. The draft decision presented in document A68/51 should acknowledge that the new emergency response programme would be directly accountable to the Director-General. Staff and resources must be flexible and mobile, not tied to specific country offices or regions, and under the direct control of the Director-General. Lastly, when establishing new emergency management structures, WHO should give qualified crisis managers the lead and be sure to consult them on the future direction of its emergency reform.

Ms GODIN (Canada), welcoming the plan to establish a global health emergency workforce, expressed support for the plan to strengthen pre-deployment and readiness through rosters and quality assurance measures, training and simulation exercises. She urged WHO to continue to address its weak operational platform as revealed in the Ebola outbreak, including in the areas of contracting mechanisms, insurance and medical evacuation methods. WHO should also implement its plan to identify a reserve corps of surge-ready staff members. Her Government supported all aspects of the plans for a contingency fund and echoed Germany's call for reports and evaluations to gauge its effectiveness. Regarding research and development, she recalled that the unprecedented speed in which potential Ebola vaccines and therapies had been brought to clinical testing had been a direct result of investment prior to the outbreak. Accordingly, Canada supported the very ambitious proposal for a blueprint for research and development in epidemics or health emergencies where preventative and curative solutions were lacking. The proactive establishment of target product profiles and the development of robust preclinical models would also help to better anticipate new drug development.

Ms PRICE (United Kingdom of Great Britain and Northern Ireland) said that the global health emergency workforce and the contingency fund would enable WHO and its partners to intervene more effectively in health crises and prevent them from escalating. It was vital that the triggers for the release of the contingency fund, which should only be used for a maximum of three months, be set at a level that both enabled action in order to avoid an emergency and prevented the use of the fund where no risk of escalation had been detected. Work on setting the triggers should be done in partnership with the Office for the Coordination of Humanitarian Affairs. Under WHO's coordination, disbursement of the funds should be done by the best-placed partner. She reiterated her country's pledge to contribute up to 10 million US dollars to the fund. She welcomed the priority afforded to strengthening the existing mechanisms within the Global Outbreak Alert and Response Network and the foreign medical teams as part of the ambitious plans for the global health emergency workforce.

Ms COOK (United States of America), while welcoming the contingency fund proposal, said that the main problem during the early Ebola outbreak response had been a lack not of resources, but of organizational structure. Consideration of WHO's initial performance had revealed that it needed to be able to make funds available early to improve the prospects of halting a health emergency, rather than delaying the activation of funds until an outbreak had become an epidemic. Thus it was essential to examine the triggers for the activation of such funds. In addition, the contingency fund should be reserved for the first phase of the response and available for up to three months from the onset of a health emergency. She commended the review of existing mechanisms in order to assess the fund's pertinence in relation to those and recommended that WHO report on the uses of the fund after two years so that Member States could evaluate its usefulness. In addition, the competent authorities for activating and distributing the funds should be specified, the more so as fragmentation of authority had hindered WHO's Ebola response. Sustainability was another important issue. The United States supported a replenishment model, and would also endorse the provision by WHO itself of flexible initial funding to help set up the contingency fund.

Mr KONG Insik (Republic of Korea) supported the establishment of the contingency fund as the outcome of a lesson learned from the tardy response to the outbreak. Mechanisms for its transparent use and for additional fundraising should also be considered. It was crucial to set up domestic and international surveillance systems with a view to preventing further outbreaks of infectious disease and providing a basis for research and risk assessment. The timely exchange of information with respect to quarantine and disease should also be promoted, in line with the International Health Regulations (2005). He expressed the hope that the global health security agenda would provide an effective policy platform applicable to both affected and non-affected countries.

Dr EVANS (World Bank) welcomed the probing analysis and practical recommendations of the Ebola Interim Assessment Panel and called for a well-funded Organization that could support countries as they prepared to meet the challenges of increased global interdependence and shared vulnerability. In the light of WHO's chronic underfinancing resulting from the zero-nominal growth policy, he urged all Member States to reconsider that policy which endangered all WHO's core functions in the long term. The establishment of a contingency fund constituted a critical element of rebuilding the financing architecture for pandemic risk management. The World Bank Group in cooperation with private sector partners was developing a global pandemic emergency financing facility which would swiftly disburse resources upon receipt of a trigger from WHO, to support the rapid deployment of personnel during outbreaks. The funding facility would thus complement the contingency fund. Lastly, he welcomed the plan to implement the global health emergency workforce. It would be vital to determine how the recommendations of the Interim Assessment Panel, the United Nations Secretary-General's High-Level Panel on the Global Response to Health Crises, and other review bodies, could be swiftly and sustainably financed through existing and new mechanisms. He drew attention to several upcoming forums on pandemic financing.

Dr Song-En HUANG (Chinese Taipei) supported the proposals for a contingency fund and the plan to expand and strengthen the global health emergency workforce to fight future outbreaks. Chinese Taipei would contribute to strengthening global preparedness for, and prevention of, emerging infectious diseases, and to rebuilding essential health systems in countries affected by the Ebola virus disease, by sharing its experiences in health communications and electronic disease surveillance systems. Chinese Taipei would be hosting Ebola virus disease safety training courses to further strengthen skills of health care personnel in line with the International Health Regulations (2005).

Mr WEBB (United Nations Development Programme) said the Ebola virus disease outbreak had demonstrated the need for crisis prevention mechanisms in all health systems. The failure to pay salary and incentives to health care workers could have been prevented, had robust governance systems been in place, and the UNDP Payment of Ebola Workers Programme, set up to alleviate grave payment difficulties, would provide relevant lessons for future health and development crises. Epidemic preparedness must be focused on health systems, not health sectors, and should include health worker payment systems as a priority. To that end, WHO and other partners would be able to facilitate the appropriate inter-agency coordination, inter-institutional data creation and sharing protocols, country-based needs assessment protocols and technical assistance. Moreover, development gains made during the current Ebola virus disease crisis should not be lost, including banking and payment systems. He noted that the proposed contingency fund would emphasize preparedness, prevention and response, and provide funding for health care workers, including hazard pay and insurance. The response to future health crises required robust investment in social mobilization and community support; strengthened engagement and coordination across the health sector; and the inclusion of other sectors such as communication, banking and transport.

Ms VIVIANI (United Nations Children's Fund (UNICEF)), recalling UNICEF's long-standing collaboration with WHO, said that UNICEF had worked with WHO to mobilize, engage and educate communities with regard to the Ebola virus disease; and had provided water, sanitation, hygiene and nutrition, and had ensured the supply of key commodities and medicines. While lessons had been learned, further evaluations would help to inform future responses, and UNICEF looked forward to WHO's continued technical leadership and coordination in public health emergencies.

Dr BJORO (International Council of Nurses), speaking at the invitation of the CHAIRMAN, said that the falling number of cases of Ebola virus disease did not mean the end of the commitment to end outbreak. She commended the work of WHO, with the support of others, in rebuilding and developing health systems. Adequate numbers of trained nurses were required for the prevention and control of infections and for ensuring patient safety, and she urged WHO to include longer-term health workforce planning and continuing education of health care professionals in efforts to strengthen health systems. Finally, she called on WHO and its Member States to take into account the long-term social impact of the Ebola virus disease outbreak, which included orphaned children of health care workers and stigmatization.

Ms KUSYNOVÁ (International Pharmaceutical Federation), speaking at the invitation of the CHAIRMAN, said that the Ebola virus disease outbreak had revealed the vulnerability of health systems, and the need to prioritize education on hygiene and prevention strategies and the protection of health care professionals. The International Pharmaceutical Federation recommended the inclusion of pharmacists in crisis plans, to facilitate timely access to medicines and evidence-based information and to assist in first response, triage, immunization and first aid. It had developed tools for pharmacists to act against the Ebola virus disease or other infectious diseases and communicate medication instructions in situations where there was no common language. She welcomed ongoing research into new medicines, which would facilitate continuing progress.

Ms NWACHUCWU (GAVI Alliance), speaking at the invitation of the CHAIRMAN, said that according to preliminary estimates, immunization coverage had dropped by 20% to 40% in the three Ebola-affected countries, and outbreaks of diseases such as measles were being observed. Thousands of children were at risk due to disruption of routine immunization programmes. The GAVI Alliance was prepared to invest significantly in restoring confidence in the public health system and ensuring availability of future Ebola vaccines, and would provide up to US\$ 80 million to assist with health and immunization system recovery efforts, including the doubling of health system strengthening support to the affected countries. Immunization recovery plans should be rapidly implemented in each country to prevent further deaths and disability. The need to rebuild confidence in public health systems should not be underestimated. The Ebola crisis had highlighted the weaknesses of health systems and an effective global emergency response was needed for the future.

Mrs BURDET (Medicus Mundi International and the People's Health Movement), speaking at the invitation of the CHAIRMAN, said that if the Ebola outbreak did not trigger substantial investments in building resilient health systems, pre-existing deficiencies would be exacerbated. The emphasis in EBSS/3/INF./2 on the need to ground health systems in primary health care and universal health coverage principles, rather than create a vertical programme, was welcome. However, it was deplorable that the African Public Health Emergency Fund established by the WHO Regional Office for Africa had had no financing during the Ebola crisis, and that the African Development Bank had refused to support it. WHO's presence in the Region would be further undermined if the African Centre for Disease Control and Prevention was established without the involvement of the Regional Office. A concerted and coordinated effort among Region members was needed to properly address the health challenges revealed by the Ebola crisis. She did not share the Secretariat's appreciation of the pharmaceutical industry for its development and roll-out of Ebola vaccines, blood therapies, medicines and diagnostics. The prevailing model of profit-driven research and development had neglected Ebola since the isolation of the virus in 1976, and an alternative model that delinked research and development from profits and provided more affordable treatment was needed.

Ms DIJK (International Federation of Medical Students' Associations), speaking at the invitation of the CHAIRMAN, said that prior to establishing the global health emergency workforce, WHO should ensure that its deployment would be based on transparent and scientific mechanisms supported by concrete guidelines for its deployment, and consider whether it could be used in situations not involving communicable diseases. Clear and transparent financing mechanisms for the workforce were needed at global and local levels, and it was imperative to identify how they would support local organizations. WHO should determine how the workforce would be integrated with the International Health Regulations (2005) and previous World Health Assembly resolutions on human resources for health, to avoid duplication of efforts and waste of resources. It was important to tackle the most basic social determinants of health and invest in health literacy to proactively prevent outbreaks. She thanked WHO for recognizing the role of young people as first responders in the crisis, and said that the Kick Ebola Out campaign organized by members from Sierra Leone and Guinea was a prime example of how medical students could support the emergency workforce.

Dr KAMAL-YANNI (OXFAM), speaking at the invitation of the CHAIRMAN, welcomed the findings of the Ebola Interim Assessment Panel and the recommendation on strengthening WHO's role as the global lead agency in health emergencies. However, strong accountability mechanisms targeting WHO's performance needed to be established. To fulfil its role, WHO required full political and financial support, and its current funding model had to shift from voluntary but earmarked contributions to adequate, flexible and predictable contributions from Member States. Strengthening the role of communities was essential to successfully controlling outbreaks. Governments should recognize that long-term investment in building resilient health systems would protect national and international health, and would entail investment in an adequately-sized and trained health workforce;

available medical supplies; robust health information systems; sufficient well-equipped health facilities; sufficient financing from domestic resources; and a strong public sector to deliver equitable quality services.

Ms DURLING (World Vision International), speaking at the invitation of the CHAIRMAN, said that further collaborative action was needed to get to zero cases and ensure recovery from the outbreak. Global surveillance, prevention and response mechanisms were ineffective without functioning health systems and active community participation. The Ebola outbreak had exposed the vulnerability of weak health systems and the significant risk they posed. But it was well-known that prevention was more effective than cure, certainly more effective than emergency response. Member States and the Secretariat should prioritize social mobilization and comprehensive support to weak health systems as part of preparedness plans to prevent future large-scale disease outbreaks and emergencies with health consequences.

Dr AYLWARD (Assistant Director-General), responding to the various comments, acknowledged the strong support expressed for the Director-General's plan for a global health emergency workforce, the options for a contingency fund, and the proposals for advancing research and development in the context of emergencies and outbreaks of infectious diseases. The Secretariat had noted the call that all of that needed to be part of a larger reform, in that all emergency work within the Organization needed to be coordinated, with all financial and human resources deployed in accordance with humanitarian principles, and that all such work needed to be aligned within the wider international emergency architecture, including the United Nations system, civil society and humanitarian organizations. It had also noted that support would be contingent on clear, unambiguous and professional emergency management and leadership under the Director-General, which should be informed by emergency management experts from other organizations. The Director-General was establishing an advisory group to guide the process in that regard.

Regarding the global health emergency workforce, he assured delegates that it would be built on existing structures, such as the GOARN Network, Global Health Cluster and the foreign medical teams that had played a vital role in the response to Ebola and other crises, and that the Director-General's plan would provide a more explicit focus on the need to build the capacity of national responders. Key groups would be appropriately represented on a workforce steering committee. In operationalizing the plan, the Secretariat would furthermore take into account the need to establish a common operational support platform; to increase the capacity of WHO, focusing in particular on its added value in coordination and technical support on the ground; and to ensure that staff were sufficiently flexible and geographically mobile to be utilized to best effect in the new emergency programme.

Turning to the contingency fund, he thanked the Member States that had made initial pledges and, responding to the specific question of triggers and the balance between early activation of the fund and its use to manage risks without its being rapidly depleted, he said that attention would be paid to the grading of risks and crises within the new emergency response framework currently being assembled for consideration by the Director-General and the Global Policy Group, which should provide a mechanism capable of ensuring that balance. As to interaction with other funds, the contingency fund had been carefully designed to complement those that facilitated the procurement of equipment and supplies for large-scale projects, for example, but which did not cover staffing needs for surge support. More details would be provided as the specifics of the fund's modus operandi were further developed. Meanwhile, the Secretariat would take into account the request that the fund be used for an initial three- to six-month period in order to allow enough time to launch appeals to bring other financing mechanisms into play. Regarding the call for the contingency fund to absorb other WHO funds, it was important to bear in mind that they covered critical gaps beyond the initial three- to six-month period, especially in protracted crises, and that any consolidation of funds must not result in a loss of flexibility in their utilization at the global and regional levels. He suggested that

consolidation might be delayed until after the pilot period. As to the sustainability of the fund, the Secretariat had noted the suggestions made in regard to its capitalization from voluntary contributions, existing WHO funds and even the private sector, which would give rise to another set of issues, including those relating to replenishment.

Responding to the call for a blueprint for future research and development agendas, he said that having such blueprints was central to the Director-General's vision in regard to the Organization's work in infectious disease risk management. One delegate had expressed a strong preference for WHO to focus on coordinating rather than conducting research and that, too, was consistent with current thinking within the Organization. In the case of the Ebola crisis, however, WHO had been compelled to intervene as "trialler of last resort" to tackle critical gaps in the clinical trial agenda, when other agencies had been unwilling to step forward. While it might well have to do so again in the event of other outbreaks of infectious diseases, he assured delegates that it had not been seen as precedent-setting.

Dr SMITH (Executive Director, Office of the Director-General), responding to the question raised by the delegate of Egypt on the sustainability of the contingency fund, said that the fund would be financed through a replenishment model that drew on voluntary contributions, adding that multi-year agreements with Member States could help to ensure a measure of sustainability. Furthermore, the fund was to be established by the governing bodies and, hence, would be sustained by the political commitment reflected in that status; its replenishment would tie in with the financing dialogue for the biennial programme budget; and, in accordance with the prevailing accountability mechanism, there would be regular reports to the Member States on the current assets, expenditures and results achieved. As such, it had a strong chance of being able to continue, over time, to provide the Organization with adequate funding for its work in emergency situations.

The CHAIRMAN drew attention to the draft decision contained in document A68/51 and informed the Committee that the Programme, Budget and Administration Committee of the Executive Board had proposed that a drafting group, co-facilitated by the delegates of South Africa and the United States of America, be set up to finalize the text. Should the Committee agree to the proposal, the delegations that had commented on the draft decision would be invited to take up their comments in the drafting group and the Committee's consideration of the item would be suspended while the drafting group met.

It was so agreed.

The meeting rose at 18:40.

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